



OFFICIAL

Flexible Endoscopic Evaluation of Swallowing (FEES) Guideline

1. Applicability

Site	Operational Area	Applicable to
Armada le Health Service	Speech Pathology Intensive Care Unit (ICU)	Speech Pathologist Medical - ICU

2. Purpose

Flexible endoscopic evaluation of swallowing (FEES) is an instrumental swallowing assessment technique used to evaluate and manage dysphagia.

This guideline outlines the standards required for conducting a Speech Pathology led Flexible Endoscopic Evaluation of Swallowing (FEES) service in the hospital and outpatient settings at Armada le Health Service (AHS). Observance to this policy ensures that FEES procedures are performed safely, accurately and in accordance with evidenced based practice and that site-specific regulatory guidelines are maintained.

3. Guidelines

FEES involves passing a nasendoscope/ rhinolaryngoscope transnasally to a position that allows visualisation of the hypopharynx and larynx. Benefits of FEES to improve patient care include:

- Early diagnostics of dysphagia
- Avoid medical complications due to dysphagia
- Support earlier return to oral intake in high-risk patient groups
- Positive impact on tracheostomy weaning

3.1 Indications for use

- Assessing management of secretions
- Comprehensive evaluation of swallow function, swallow safety (airway protection/aspiration), and swallow efficiency (residue after the swallow)
- Assessing pharyngeal sensation
- Evaluating the effectiveness of interventions
- Biofeedback/teaching in the management of dysphagia
- Visualising laryngopharyngeal surface anatomy and mucosa
- Assessing patients swallowing who are unable/unsuitable for videofluoroscopy

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

3.2 Patient Suitability

Possible contraindications for FEES due to scoping risks include the following:

- Unstable cardiac conditions
- Skull base/facial surgery or fracture within the previous six weeks
- History of severe epistaxis
- Trauma to nasal cavity secondary to surgery/injury within the previous six weeks
- Active stridor (until reviewed by ENT)
- Sino-nasal and anterior skull base tumours/surgery
- Nasopharyngeal stenosis
- Craniofacial anomalies
- Hereditary haemorrhagic telangiectasia
- Choanal atresia

Proceed with caution (high risk):

- Limited pharyngeal or laryngeal space
- Significant airway limitation due to the presence of large-volume disease, such as cancer
- Severe movement disorders and/or severe agitation
- Vasovagal history
- Bleeding risks

3.3 Consent

Consent needs to be obtained from the patient and treating medical team if an inpatient. If the patient is an outpatient, then GP or managing outpatient physician needs to provide consent, with any risks explained and discussed if relevant to the patient. The Speech Pathologist obtaining consent is required to explain the procedure in lay terms and warn of risks and benefits associated with the procedure in verbal and written forms. Common risks include discomfort passing the nasendoscope. Uncommon risks include epistaxis, vasovagal, and laryngospasm.

Patient and medical consent needs to be documented in the DMR.

4. Process

The AHS Speech Pathology department independently manage the referral and assessment for FEES. They also accept referrals for FEES from Medical staff.

Speech Pathologists performing nasendoscopy and interpreting FEES require specific competency training and credentialing.

4.1 Referral

The managing Speech Pathologist will:

- Complete clinical bedside examination (case history, oro-motor exam, diet & fluid trials)
- Consider indications and rationale for FEES vs other instrumental assessment/management
- Discuss case / appropriateness for FEES with FEES Senior SP
- Consider contraindications and discuss with the Medical Team

- Obtain patient/NOK & Medical Team consent and document in DMR
- Arrange time to complete FEES
- Alert ward staff of FEES appointment, clarify patient needs to be upright and any specific seating requirements, cessation of non-oral feeding and fasting not required.

4.2 Infection & Prevention Control Procedures

In accordance with AHS site policy, a FEES examination is classified as an aerosol generating procedure and thus Airborne Precautions apply. Clinicians conducting FEES examinations will adhere to hand hygiene standards and wear appropriate personal protective equipment (PPE) including:

- N95 mask
- Protective eyewear
- Apron/gown – based on risk assessment of risk to clinician or specific to patient (i.e. high secretion load suggest wearing long sleeved gown)

All surfaces of equipment to be cleaned with a combined detergent-disinfectant wipe post-examination. All disposable equipment to be disposed of correctly in patient rooms.

4.3 Equipment

The following equipment is required to perform this procedure:

- C-MAC monitor 8404ZX on medical trolley
- Karl Storz CMOS disposable Video Rhino-Laryngoscope
- Blue & green food dye
- PPE (N95 masks, goggles, gown/apron, gloves)
- Lubricant jelly sachet
- Detergent-disinfectant wipes
- Oral trials as indicated - ice-chips, L0 thin water (dyed blue), L0 thin milk (hilo/skim), L4 puree diet (dyed green), other diet/fluid trials as indicated

4.4 Assessment

Conduct FEES assessment in line with [Procedure and protocol guidelines for flexible endoscopic evaluation of swallowing - FEES - AH and KH.docx](#).

4.5 Risk Management

Risks will be mitigated by appropriate patient screening and identification of contraindications. Low rate of complications are generally reported with FEES examinations, most commonly minor discomfort on nasendoscope insertion and gagging.

If any serious adverse event occurs during FEES procedure, cease procedure and contact relevant medical officer. Notify Speech Pathology Coordinator, document event in DMR, and submit Clinical Incident report via CIMS.

As there is no onsite ENT at AHS, the below adverse event risk management matrix is proposed for minor complications.

Risk		Management Plan
Common	Discomfort or tickle upon nasendoscope insertion	• Deep breaths through nose • Gentle encouragement & reassurance by SP • Consider using paediatric nasendoscope if tight nasal passage (if available)
Uncommon	Feeling of light headedness, sweaty, bradycardia, and/or loss of consciousness (vasovagal)	• Remove nasendoscope & cease assessment • Medical review as indicated
	Nose bleeds (epistaxis)	IF bleeding occurs – • Sit patient up, head forward, constant pressure nasal alae (occluding both nares) for 10-15 minutes • Emesis bag under nose • Ice to forehead/neck • Reassure patient • Medical review as indicated
	Spasm of the vocal cords (laryngospasm)	• Remove nasendoscope & cease assessment • Instruct patient on relaxed throat breathing • Gentle sniff IN through nose • Gentle blow of air OUT through tight pursed lips (like breathing out a straw) • Reassure patient • Medical review as indicated • To call duty anaesthetist/MET call if severe (decreased O2 saturation/airway compromise not resolving)

5. Documentation

The managing Speech Pathologist is responsible for documenting a summary of findings and/recommendations in the patients' DMR immediately post procedure.

The Endoscopist Speech Pathologist is responsible for compiling a formal FEES report.

Where indicated, the Endoscopist will have ENT review images of anatomy requiring further comment in scheduled meetings. The Endoscopist will then document diagnostic findings and recommendations in patient DMR and arrange follow up as indicated.

6. Roles and responsibilities

The following outlines the specific roles and responsibilities relevant to this procedure:

Role	Responsibility
AHS Speech Pathology	Independently manage FEES referral & assessment, including passing of the nasendoscope, swallow assessment & feedback
AHS ICU	Onsite medical governance and support of adverse events
RPH ENT	- Remote review of images of altered anatomy requiring diagnosis/comment and management plan - If urgent ENT advise required, on site managing medical team to liaise with RPH ENT Registrar on call for support (i.e. concerns of airway compromise or malignancy)

7. Compliance monitoring

The AHS site lead FEES Clinician in consultation with the Coordinator of Speech Pathology will be responsible for general oversight, compliance, and efficacious implementation of FEES at AHS. This may include the following:

- FEES networking meeting – attendance & engagement
- Inter-rater meetings regularly within AHS SP
- FEES Service audits
- QI projects
- Investigation of any adverse events

8. Policy context

The following documents are required to give effect to this procedure:

- HDWA
 - [Clinical Incident Management Policy](#)
- AKG
 - [Cleaning and Disinfection of Reusable Clinical Non-Invasive Equipment Procedure](#)
 - [Hand Hygiene Policy](#)
 - [Standard and Transmission Based Precautions Procedure](#)
 - [Recognition and Response to Acute Deterioration Policy](#)

9. Supporting information

The following documents support the implementation of this procedure:

- [Speech Pathology Australia \(SPA\) Clinical Guideline 2019](#)

- [American Speech-Language-Hearing Association \(ASHA\) Practice Portal 'Flexible Endoscopic Evaluation of Swallowing' \(FEES\)](#)
- [Royal College of Speech Language Therapists \(RCSLT\) UK Position Paper 2021](#)

10. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.01
- Standard 2 – Action 2.01
- Standard 3 – Action 3.01
- Standard 5 – Action 5.01
- Standard 6 – Action 6.01
- Standard 8 – Action 8.01

11. References

1. Langmore, S.E. (2017) History of Fiberoptic Endoscopic Evaluation of Swallowing for Evaluation and Management of Pharyngeal Dysphagia: Changes over the Years. *Dysphagia*. - [History of Fiberoptic Endoscopic Evaluation of Swallowing for Evaluation and Management of Pharyngeal Dysphagia: Changes over the Years](#)
2. Langmore, S.E., Scarborough, D, Kelchner, L., Swigert, N., Murray, J., Reece, S., Cavanagh, T., Harrigan, L., Scheel, R., Gosa, M., & Rule, D. (2022) Tutorial on Clinical Practice for Use of the Fiberoptic Endoscopic Evaluation of Swallowing Procedure With Adult Populations: Part 1. *American Journal of Speech-Language Pathology*. 31. Pp. 163-187. [langmore-et-al-2022-tutorial-on-clinical-practice-for-use-of-the-fiberoptic-endoscopic-evaluation-of-swallowing \(1\).pdf](#)

12. Glossary

The following definitions are relevant to this procedure:

Term	Definition
Dysphagia	Difficulty or discomfort in swallowing
Flexible Endoscopic Evaluation of Swallowing (FEES)	Thin, flexible endoscope is passed through the nasal passage into the pharynx to evaluate swallowing function
Videofluoroscopy	Real-time moving X-ray procedure used to evaluate the mechanics of swallowing
Epistaxis	Nose bleeding
Vasovagal	Feeling of light headedness, sweaty, bradycardia, and/or loss of conscious
Laryngospasm	Spasm of the vocal cords

13. Document contact

Director Allied Health & Ambulatory Care

Enquiries relating to this procedure may be directed to:

Title: A/Coordinator - Speech Pathology
 Department: Allied Health
 Email: Julie.Varian@health.wa.gov.au

14. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V1	16 03 2026	16 03 2027	New document

15. Authorisation

This procedure has been authorised and issued by:

Authorised by	Leanne Watson- A/ Director Allied Health & Ambulatory Care
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